



DoctaRx

Connecting care. Empowering communities.

PREPARED FOR FEDERAL MINISTRY OF HEALTH AND SOCIAL WELFARE

DoctaRx Abuja Pilot Program

AI-enabled digital health coordination platform for a controlled FCT pilot.



DoctaRx
healthcare coordination operating layer

DoctaRx coordinates the healthcare journey

The problem is fragmented handoffs. The solution is one coordination layer for care access, clinical workflow, referrals, analytics, and reporting.

Problem

Care patients often split across multiple consultation, diagnosis, referral, and reporting.

Solution

DoctaRx connects those steps into a digital operating layer across existing care teams.

Visibility

Understanding one aggregate operational, insurance, time, follow-up, and service pressure.

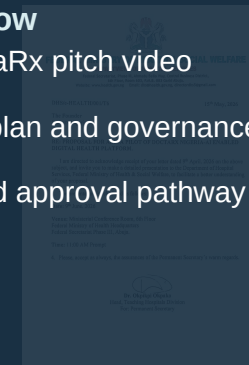
Proof point

The Abuja/FCT program tests workflow fit before any broader scale decisions.

DoctaRx answer: connect the care journey, make handoffs visible, and turn activity into actionable health-system insight.

Presentation flow

- Elevated DoctaRx pitch video
- Detailed pilot plan and governance
- Discussion and approval pathway





WHY WE ARE HERE

A detailed response to the Ministry invitation

Meeting purpose

- Clarify the DoctaRx proposal
- Show platform workflow and pilot model
- Agree next steps for a controlled Abuja/FCT pilot

What is in scope

- PHC-supported intake
- Provider review and video consultation
- Prescription, referral, follow-up, dashboard and reporting readiness

What is not claimed

- No signed approval is claimed
- No real patient data is shown
- No national rollout before pilot evidence



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VISION

DoctaRx as the care coordination operating layer

For patients

- Simpler request capture
- Visible next steps
- Better follow-up support

For clinicians

- Structured review
- Video consult where appropriate
- Clear documentation path

For leadership

- Aggregate activity dashboard
- Forecasting signals
- NHMIS/DHIS2-ready data pathway



PROBLEM

Where care coordination breaks down today

Fragmented access

- Patients often do not know where to start
- PHCs lack fast escalation options
- Remote doctor support is inconsistent

Weak follow-up visibility

- Referrals are hard to track
- Prescriptions are disconnected
- Missed follow-ups are invisible

Limited operational data

- Facility leaders see late reports
- Planning demand is hard
- Digital reporting formats vary



Why Abuja/FCT is the right controlled starting point

Controlled scope

- 2-3 PHCs
- One referral pathway
- Defined users and support desk

Measurable outcomes

- Consultation completion
- Response time
- Referral closure and follow-up visibility

Scale evidence

- Workflow fit
- Cost and staffing model
- Reporting and governance lessons

OPERATING WORKFLOW

From patient request to Ministry visibility

The pilot tests a practical care journey, not a loose technology demo.

1

Request

Patient request captured at PHC or portal.

2

Intake

PHC officer confirms basic context and routing.

3

Consult

Doctor reviews and conducts video consultation where appropriate.

4

Care action

Notes, prescription, referral, or follow-up are documented.

5

Referral

Hospital or specialist pathway receives a structured summary.

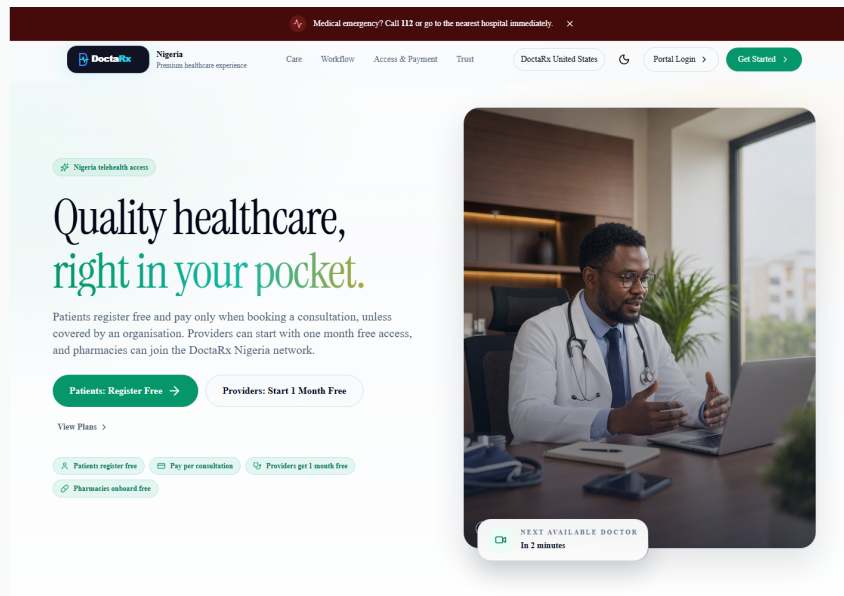
6

Dashboard

Leadership sees aggregate indicators and monthly reporting.

Real DoctaRx surfaces support the story

Screens are used to show the actual product direction while demo data protects privacy.



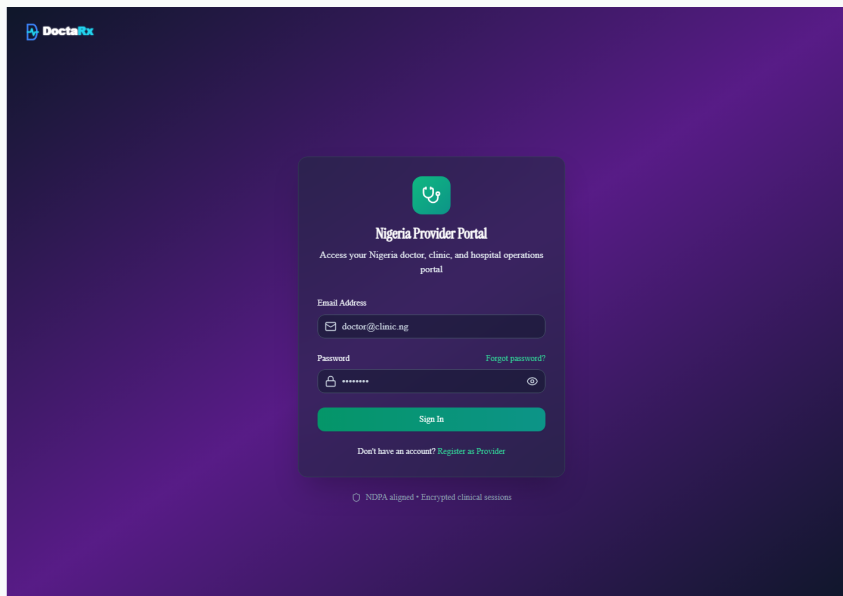
What this proves in the room

- Patient and provider routes are visible
- Walkthrough demonstrates click actions
- Demo-only information is used
- Generated assets fill unavailable feature states

PROVIDER WORKFLOW

Doctor review, consultation, and care actions

The clinical workflow is framed around review, notes, prescription, referral, and follow-up.



What this proves in the room

- Provider portal route exists
- Workflow will be configured for selected facilities
- Role-based access will govern real usage
- Audit and support model will be approved

VIDEO CONSULTATION

Remote clinical reach inside the DoctaRx workflow

The video-call segment uses live-motion demo tiles and does not show patient-level data.



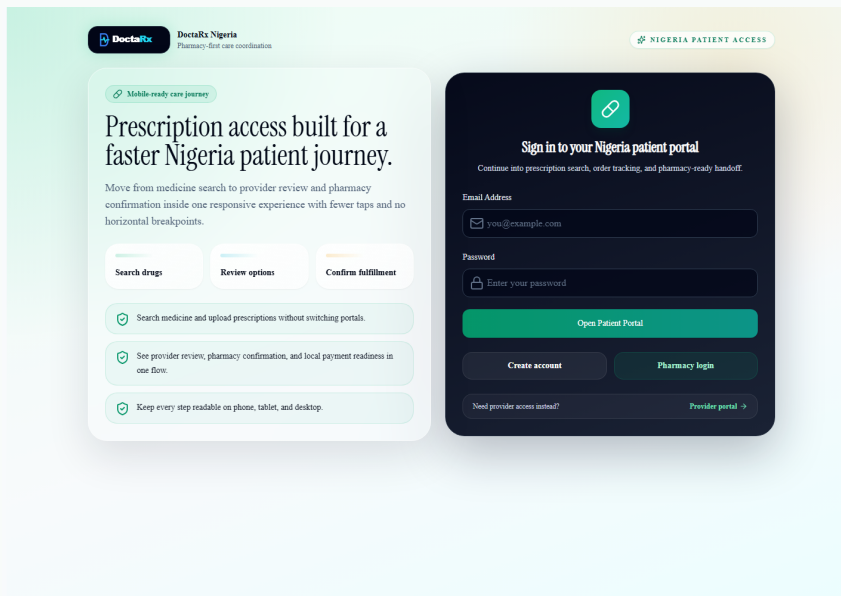
What this proves in the room

- Live consultation context
- Care actions sit beside the call
- Supports junior doctor and consultant workflow
- Tied to notes, referral, prescription, and follow-up

PATIENT WORKFLOW

Patient request to follow-up visibility

The patient-side journey remains simple while facility teams carry the clinical routing.

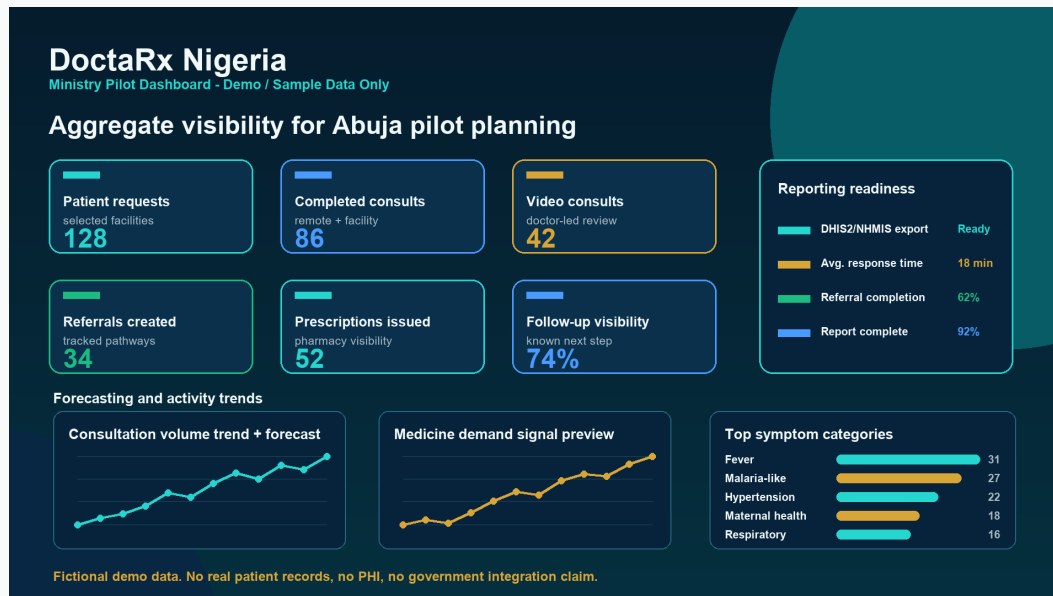


What this proves in the room

- Request capture
- Status and next step visibility
- PHC-supported workflows where needed
- No sensitive data appears in public demos

Aggregate insight for planning

Pilot activity becomes non-identifying operational visibility for review meetings.



What this proves in the room

- Consultation and referral trends
- Response-time tracking
- Medicine-demand signal
- Facility workload and follow-up visibility



Reporting will be agreed before scale-up

Data elements

- Define indicator dictionary
- Map facility and service fields
- Separate aggregate from patient-level views

Approval workflow

- Review data definitions
- Approve monthly report pack
- Keep audit trail for generated reports

Integration path

- Dry-run exports first
- Validate against Ministry format
- Move to DHIS2 only after governance approval



The pilot proceeds with clear guardrails

Access control

- Role-based users
- Least privilege
- Separate clinical and leadership views

Data protection

- NDPA-aligned privacy review
- Consent model
- No public PHI exposure

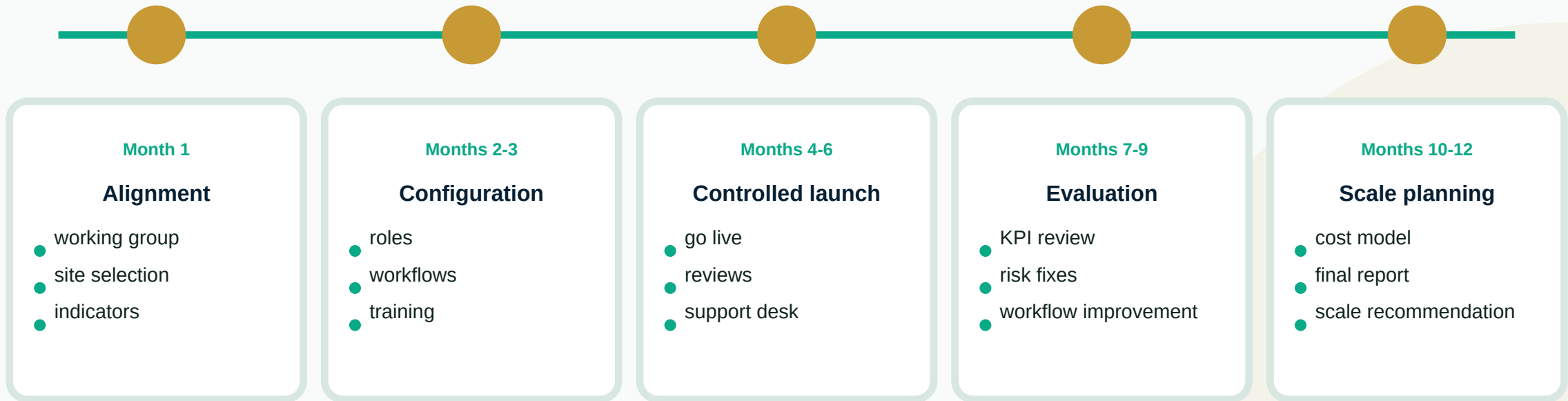
Operational assurance

- Audit logs
- Support escalation
- Backup and downtime procedure

IMPLEMENTATION TIMELINE

12-month pilot roadmap

A phased Abuja pilot designed for approval, controlled launch, evaluation, and scale decision.





EVALUATION FRAMEWORK

Pilot success metrics

We will run the pilot with measurable, reviewable indicators from day one.

Consultations

Completed remote/facility-supported reviews

Target trend: steady adoption by month 3

Response time

Request capture to first clinical response

Target trend: measurable reduction from baseline

Referral closure

Referrals with documented endpoint

Target trend: higher closure visibility

Follow-up visibility

Cases with known next-step status

Target trend: 70%+ by controlled launch

Reporting completeness

Pilot events represented in dashboard

Target trend: 90%+ weekly completeness

User adoption

PHC, doctor, pharmacy, and Ministry usage

Target trend: active use across selected sites



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OPERATING GOVERNANCE

Who owns what during the pilot

CI	Ministry / DHS	Approval, oversight, focal persons, review cadence
	Facilities	PHC intake, referral pathway participation, frontline feedback
	Clinicians	Clinical review, documentation, prescriptions, referral decisions
	Pharmacy partners	Prescription visibility and fulfillment status workflow
	DoctaRx	Configuration, training, support, dashboard, reporting pack
	Data governance lead	Consent model, role access, audit review, reporting approvals

BUDGET LOGIC

Use-of-funds categories for a credible pilot

We will finalize numbers after sites, users, support level, devices, and reporting requirements are confirmed.

 Pilot execution	facility setup, training sessions, supervision, review meetings
 Product and engineering	portal configuration, workflow tuning, dashboard, audit features
 Connectivity and resilience	devices, backup power options, cloud, data hosting, support tools
 Local operations	pilot manager, support desk, facility onboarding, partner coordination
 Governance and compliance	NDPA/privacy review, role permissions, consent and audit procedures
 Reporting and evaluation	baseline capture, KPI packs, monthly reports, final assessment
 Contingency	inflation, transport, replacement devices, unexpected support load



RISKS AND MITIGATIONS

Questions the room is likely to ask

Adoption risk

- Start with few facilities
- Train by user role
- Bi-weekly review and workflow fixes

Connectivity risk

- Device and data plan assessment
- Offline fallback SOP
- Support desk and escalation

Governance risk

- Written indicator approval
- Demo-to-production data separation
- Audit and privacy review before go-live



How the pilot becomes a durable service

Institutional fit

- DHS oversight
- Facility focal persons
- Monthly reporting cadence

Operating model

- Local support team
- Training-of-trainers
- Clear cost per facility/user

Scale criteria

- Evidence of adoption
- Improved follow-up visibility
- Approved reporting pathway



DECISION PATHWAY

Approve a controlled pilot, not an uncontrolled rollout.

The next step is a technical working session to agree facilities, indicators, governance, training, support, and reporting format.

- Confirm pilot focal persons
- Select 2-3 PHCs and referral pathway
- Approve indicators and reporting pack
- Schedule onboarding and controlled launch

Presentation close

DoctaRx is prepared to work under Ministry guidance, with demo-only data in this presentation and a controlled implementation model for any real pilot activity.